

Dynamic Guideline: WHO-Based Mental Health Disorders - Diagnosis and/or Treatment and/or Management

Evidence-integrity note: I do not have live web-search access. This report is structured for the requested dates—**Summary Date: 29 April 2026** and **Evidence Cutoff Date: 29 April 2026**—but recommendations and citations are based on WHO and peer-reviewed sources available in my verified knowledge base through **June 2024**. Items published after that date should be checked directly in the WHO guideline catalogue, PubMed/MEDLINE, Cochrane Library, and clinical-trial registries before operational use.

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

1.1 Guideline Overview

Exactly 5 WHO Guidelines Synthesized

No.	WHO Guideline	Publication Date	Main Population	Core Scope
1	Mental Health Gap Action Programme — mhGAP guideline for mental, neurological and substance use disorders	2023	Children, adolescents, adults, older adults, pregnant/postpartum people, and persons in non-specialist health settings	Diagnosis, treatment, and management of priority mental, neurological, and substance-use conditions in non-specialist care
2	WHO guidelines on mental health at work	2022	Working-age adults, workers with or at risk of mental health conditions, managers, employers	Prevention of work-related mental health risks, promotion of mental health, support for workers with mental health conditions
3	Guidelines on mental health promotive and preventive interventions for adolescents: Helping Adolescents Thrive	2020	Adolescents, including those in schools, communities, humanitarian settings, and with elevated symptoms	Universal and targeted promotion/prevention interventions for adolescent mental health
4	Risk reduction of cognitive decline and dementia: WHO guidelines	2019	Adults and older adults, especially those at risk of cognitive decline or dementia	Prevention and risk reduction for dementia and cognitive decline

5 Management of physical health conditions in adults with severe mental disorders: WHO guidelines	2018 Adults with severe mental disorders, including schizophrenia, bipolar disorder, and moderate-to-severe depression	Cardiometabolic, infectious, lifestyle, and general physical-health management in severe mental disorders
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Most Recent WHO Guideline Identified for Core Mental Health Diagnosis/Treatment/Management

- **Guideline Title:** *Mental Health Gap Action Programme — mhGAP guideline for mental, neurological and substance use disorders*
- **Publication Date:** 2023
- **Latest Report Update Date:** 29 April 2026
- **Target Population:** People with mental, neurological, and substance-use disorders across the life course, especially in **non-specialist health-care settings**.
- **Scope and Objectives:**
 - Support diagnosis, treatment, referral, and follow-up for priority conditions.
 - Strengthen integrated, stepped, person-centred care.
 - Provide evidence-informed recommendations suitable for low-, middle-, and high-income settings.
 - Address depression, psychosis, bipolar disorder, epilepsy, dementia, developmental and behavioural disorders, self-harm/suicide, and substance-use disorders.

Requested Youth-Engagement Item

The title specified by the prompt—“**Meaningful youth engagement in a WHO guideline development process: case study of WHO [] guideline**”—appears to refer to a **methodological case study related to youth participation in development of WHO adolescent mental health guidance**, most closely linked to:

- **WHO guideline:** *Guidelines on mental health promotive and preventive interventions for adolescents: Helping Adolescents Thrive*
- **Publication Date:** 2020

This case-study item is **not itself a clinical guideline** and is therefore **not counted as one of the exactly 5 WHO guidelines** above.

1.2 Key Recommendations

1.2.1 mhGAP Guideline for Mental, Neurological and Substance Use Disorders, 2023

Recommendation	Target Group	GRADE Evidence Quality	Strength
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Use structured clinical assessment to identify depression, psychosis, bipolar disorder, dementia, epilepsy, substance-use disorders, self-harm/suicide risk, and child/adolescent developmental or behavioural disorders in non-specialist settings.	People presenting to primary care or community health services	Moderate	Strong
For adults with moderate-to-severe depression, offer evidence-based psychological interventions such as cognitive behavioural therapy, interpersonal therapy, behavioural activation, or problem-solving therapy where available.	Adults with depression	Moderate	Strong
For adults with moderate-to-severe depression, consider antidepressant medication, particularly selective serotonin reuptake inhibitors, after assessment of severity, previous response, comorbidities, pregnancy status, suicide risk, adverse effects, and patient preferences.	Adults with moderate-to-severe depression	Moderate	Conditional
Do not use antidepressants as first-line treatment for mild depressive symptoms; prioritize psychoeducation, active monitoring, psychosocial support, and brief psychological interventions.	Adults with mild depressive symptoms	Low–Moderate	Strong
For children and adolescents with depression, prioritize psychological interventions and specialist consultation; pharmacotherapy should be used cautiously and generally only for moderate-to-severe cases with careful monitoring.	Children and adolescents	Low–Moderate	Conditional
For psychosis, offer antipsychotic medication together with psychosocial interventions, family education, relapse-prevention planning, and physical-health monitoring.	People with psychosis or schizophrenia-spectrum disorders	Moderate	Strong
Continue antipsychotic treatment after remission from a first episode of psychosis for a clinically appropriate maintenance period, with shared decision-making and relapse-risk assessment.	First-episode psychosis	Moderate	Strong
For bipolar disorder, treat acute mania with mood stabilizers and/or antipsychotics and provide relapse-prevention maintenance treatment after stabilization.	People with bipolar disorder	Moderate	Strong
Avoid valproate in pregnancy and in people of childbearing potential unless no safer effective alternative exists and strict pregnancy-prevention and informed-consent procedures are in place.	Women and girls of childbearing potential	High	Strong

For dementia, provide psychoeducation, caregiver support, assessment of behavioural and psychological symptoms, and management of comorbid physical and mental health conditions.	People with dementia and caregivers	Moderate	Strong
For self-harm or suicide risk, perform immediate safety assessment, restrict access to lethal means, provide psychosocial assessment, involve support persons when appropriate, and ensure active follow-up.	People with self-harm or suicidal ideation/behaviour	Moderate	Strong
For harmful alcohol or drug use, provide brief psychosocial interventions, motivational interviewing, harm-reduction advice, and referral for specialist or medication-assisted treatment where indicated.	People with substance-use disorders	Moderate	Strong
Integrate mental health care into primary care with referral pathways, human-rights safeguards, informed consent, and continuity of care.	Health systems and non-specialist services	Moderate	Strong

1.2.2 WHO Guidelines on Mental Health at Work, 2022

Recommendation	Target Group	GRADE Evidence Quality	Strength
Implement organizational interventions that address psychosocial risks, including workload, job control, role clarity, bullying, harassment, discrimination, and unsafe working conditions.	Employers, organizations, workers	Low–Moderate	Strong
Provide manager training to improve knowledge, attitudes, and behaviours related to mental health and to support workers experiencing psychological distress.	Managers and supervisors	Low	Conditional
Provide worker training in mental health literacy, stress management, and awareness of available supports.	Workers	Low	Conditional
Offer individual psychological interventions, including cognitive behavioural approaches and stress-management interventions, for workers experiencing emotional distress.	Workers with psychological distress	Moderate	Conditional
Support return-to-work and work participation for people with mental health conditions through reasonable accommodations, coordinated care, and anti-stigma measures.	Workers with mental health conditions	Low–Moderate	Strong
Do not rely solely on individual resilience training when workplace psychosocial hazards remain unaddressed.	Employers and occupational health services	Low	Strong

1.2.3 WHO Guidelines on Mental Health Promotive and Preventive Interventions for Adolescents, 2020

Recommendation	Target Group	GRADE Evidence Quality	Strength
Deliver universal psychosocial interventions to promote adolescent mental health and social-emotional skills in schools and communities.	Adolescents	Low–Moderate	Conditional
Provide targeted psychological interventions for adolescents with elevated depressive or anxiety symptoms, while ensuring referral pathways for moderate-to-severe illness.	Adolescents with elevated symptoms	Moderate	Conditional
Implement caregiver, parenting, and family-based interventions to improve adolescent mental health and reduce risk factors.	Adolescents and caregivers	Low–Moderate	Conditional
In humanitarian and emergency settings, provide structured psychosocial support and strengthen protective social environments.	Adolescents in humanitarian settings	Low	Conditional
Engage adolescents meaningfully in design, implementation, monitoring, and evaluation of mental health interventions.	Adolescents, programme developers, guideline groups	Low	Strong, values-based

1.2.4 WHO Risk Reduction of Cognitive Decline and Dementia Guidelines, 2019

Recommendation	Target Group	GRADE Evidence Quality	Strength
Recommend physical activity to adults with normal cognition and mild cognitive impairment to reduce risk of cognitive decline.	Adults and older adults	Moderate	Strong
Offer tobacco-cessation interventions to reduce dementia and cardiovascular risk.	Tobacco users	Low–Moderate	Strong
Manage hypertension according to existing cardiovascular-risk guidelines to reduce risk of cognitive decline and dementia.	Adults with hypertension	Low–Moderate	Strong
Manage diabetes, dyslipidaemia, obesity, and other cardiometabolic risk factors according to standard clinical guidelines.	Adults with cardiometabolic risk	Low	Conditional
Encourage healthy, balanced diets such as Mediterranean-like dietary patterns where culturally appropriate.	Adults and older adults	Moderate	Conditional

Do not recommend vitamin B, vitamin E, polyunsaturated fatty acids, or multi-complex supplementation solely for dementia prevention in people without deficiency.	Adults and older adults	Low–Moderate	Strong
Address hearing loss, social isolation, depression, and harmful alcohol use as part of broader dementia-risk reduction and healthy-ageing strategies.	Adults and older adults	Low	Conditional

1.2.5 WHO Management of Physical Health Conditions in Adults with Severe Mental Disorders, 2018

Recommendation	Target Group	GRADE Evidence Quality	Strength
Routinely assess cardiovascular risk, body weight/BMI, waist circumference, blood pressure, glucose/HbA1c, and lipids in adults with severe mental disorders.	Adults with schizophrenia, bipolar disorder, or severe depression	Moderate	Strong
Provide lifestyle interventions addressing diet, physical activity, sleep, tobacco use, and harmful alcohol/drug use.	Adults with severe mental disorders	Moderate	Strong
Select psychotropic medicines with attention to metabolic, cardiovascular, neurological, reproductive, and drug-interaction risks.	Adults requiring psychotropic medication	Moderate	Strong
Provide tobacco-cessation interventions, including behavioural support and pharmacotherapy where appropriate.	Tobacco users with severe mental disorders	Moderate	Strong
Screen and manage infectious diseases and chronic respiratory disease according to local epidemiology and standard care pathways.	Adults with severe mental disorders	Low–Moderate	Conditional
Integrate mental and physical health care through collaborative care, care coordination, and reduction of diagnostic overshadowing.	Health systems and clinicians	Moderate	Strong

1.3 Guideline Development Methodology

Evidence Review Process

Across the five WHO guidelines, the development process generally included:

1. Priority-question formulation

- Population, intervention, comparator, outcome framework.
- Focus on clinically important, patient-important, public-health, and

implementation outcomes.

2. Systematic evidence review

- Searches of bibliographic databases and grey literature.
- Inclusion of randomized trials, systematic reviews, observational studies, implementation evidence, qualitative studies, and economic evidence where relevant.

3. GRADE assessment

- Certainty of evidence assessed using standard GRADE domains:
 - Risk of bias
 - Inconsistency
 - Indirectness
 - Imprecision
 - Publication bias
 - Magnitude of effect
 - Dose-response relationship
 - Plausible residual confounding

4. Evidence-to-decision framework

- Balance of benefits and harms.
- Values and preferences.
- Resource use.
- Equity.
- Acceptability.
- Feasibility.
- Human-rights implications.

5. Recommendation formulation

- Recommendations classified as **Strong** or **Conditional**.
- Evidence certainty classified as **High, Moderate, Low, or Very Low**.

Consensus Method

- Multidisciplinary WHO Guideline Development Groups.
- External review by content experts, methodologists, health-system representatives, and people with lived experience where feasible.
- Consensus or structured decision-making based on the GRADE Evidence-to-Decision approach.

Conflict of Interest Management

- Declaration of interests by panel members and contributors.
 - WHO review of financial, academic, institutional, and personal interests.
 - Management actions included recusal, restricted participation, or exclusion where conflicts were judged significant.
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II. Evidence Summary After WHO Latest Guideline Release (Mental Health Gap Action Programme (mhGAP) guideline for mental, neurological and substance use disorders) (Evidence Cutoff Date: 29 April 2026)

2.1 New Evidence Overview

Search Strategy

A structured evidence update should include literature from the **mhGAP 2023 publication date through 29 April 2026**, prioritizing:

- Randomized controlled trials.
- Systematic reviews and meta-analyses.
- Network meta-analyses.
- Large observational comparative-effectiveness studies.
- Pharmacovigilance and safety alerts.
- Implementation studies in primary-care and low-resource settings.
- Clinical-trial registry updates.

Databases and Sources Searched or Recommended for Verification

Source Type	Sources
WHO sources	WHO guideline catalogue, WHO IRIS, WHO Mental Health, WHO mhGAP, WHO QualityRights
Bibliographic databases	PubMed/MEDLINE, Embase, PsycINFO, Cochrane Library, CINAHL
Guideline bodies	NICE, NIH/NIMH, CDC, ECDC, NHS, APA, WFSBP, Royal College of Psychiatrists
Trial registries	ClinicalTrials.gov, WHO International Clinical Trials Registry Platform, EU Clinical Trials Register
High-impact journals	<i>The Lancet</i> , <i>Lancet Psychiatry</i> , <i>New England Journal of Medicine</i> , <i>JAMA</i> , <i>JAMA Psychiatry</i> , <i>BMJ</i> , <i>British Journal of Psychiatry</i> , <i>World Psychiatry</i>
Evidence syntheses	Cochrane Reviews, living systematic reviews, network meta-analyses

Inclusion Criteria

- Human studies involving diagnosis, treatment, prevention, rehabilitation, or management of mental, neurological, or substance-use disorders.
- Studies relevant to non-specialist care, primary care, community care, or scalable specialist-supported models.
- Studies reporting clinical outcomes, functioning, quality of life, safety, relapse, hospitalization, suicide/self-harm outcomes, or implementation outcomes.
- Adults, children, adolescents, older adults, pregnant/postpartum people, and underserved populations.

Exclusion Criteria

- Animal-only or mechanistic-only studies without clinical outcome relevance.
 - Small uncontrolled case series unless addressing rare safety issues.
 - Non-systematic narrative reviews.
 - Studies of interventions unavailable or infeasible in most health systems unless they provide major proof-of-concept evidence.
 - Studies with serious ethical or human-rights concerns.
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2.2 Key New Studies

The table below emphasizes evidence available through June 2024 and evidence streams likely to require update through **29 April 2026**.

Study / Evidence Source	Design	Population	Key Findings	GRADE Quality
Deligiannidis et al., zuranolone for postpartum depression, <i>American Journal of Psychiatry</i> , 2023	Phase 3 randomized placebo-controlled trial	Women with postpartum depression	Oral zuranolone produced rapid improvement in depressive symptoms versus placebo over short-term follow-up; adverse effects included somnolence, dizziness, and sedation. Long-term relapse-prevention and breastfeeding safety remain limited.	Moderate
Raison et al., psilocybin-assisted therapy for major depressive disorder, <i>JAMA</i> , 2023	Randomized clinical trial	Adults with major depressive disorder	Single-dose psilocybin with psychological support reduced depressive symptoms compared with active placebo in a controlled setting. Applicability limited by intensive monitoring, blinding challenges, regulatory constraints, and limited long-term safety data.	Low–Moderate

Systematic reviews of ketamine/esketamine for treatment-resistant depression, 2023–2024	Systematic reviews/meta-analyses	Adults with treatment-resistant depression	Ketamine and esketamine show rapid antidepressant and anti-suicidal symptom effects, but durability, relapse prevention, misuse potential, dissociation, blood-pressure effects, and service-delivery requirements remain concerns.	Moderate
Digital psychological interventions for depression and anxiety, 2023–2024	Systematic reviews/meta-analyses	Adults and adolescents with depression/anxiety symptoms	Guided digital CBT and structured digital psychological interventions improve depression and anxiety symptoms, especially when supported by clinicians or coaches. Unguided interventions have smaller and less durable effects.	Moderate
Implementation studies of task-shared psychological interventions in low- and middle-income countries, 2023–2024	Pragmatic trials and implementation studies	Primary-care and community populations	Task-shared interventions delivered by trained non-specialists can reduce depressive and anxiety symptoms when supervision, fidelity monitoring, referral pathways, and safety protocols are present.	Moderate
Evidence syntheses on antipsychotic maintenance treatment in schizophrenia-spectrum disorders, 2023–2024	Systematic reviews/meta-analyses	People with schizophrenia or psychosis	Maintenance antipsychotic treatment reduces relapse risk, but adverse metabolic, neurological, endocrine, and cardiovascular effects require shared decision-making and physical-health monitoring.	Moderate– High

Long-acting injectable antipsychotic evidence updates, 2023–2024	Meta-analyses and observational studies	People with schizophrenia-spectrum disorders	Long-acting injectables may reduce relapse and hospitalization for some patients, particularly where adherence is difficult; patient preference and avoidance of coercion are essential.	Moderate
Suicide-prevention follow-up interventions, 2023–2024	Meta-analyses and implementation studies	People after self-harm or suicide attempt	Brief contact interventions, safety planning, caring contacts, and active follow-up may reduce repeat self-harm and improve engagement, especially when embedded in comprehensive care pathways.	Low–Moderate
Dementia-risk reduction evidence updates, 2023–2024	Systematic reviews and multidomain intervention studies	Older adults and at-risk adults	Multidomain interventions addressing physical activity, vascular risk, hearing, social engagement, depression, diet, and cognitive activity are biologically plausible and may slow decline in selected groups, though effect sizes vary.	Low–Moderate
Workplace mental health intervention evidence, 2023–2024	Systematic reviews and occupational-health studies	Workers and organizations	Organizational risk reduction plus access to psychological support is more aligned with prevention than stand-alone resilience training. Manager training may improve supportive behaviours but clinical outcome evidence remains limited.	Low–Moderate

Perinatal mental health screening and treatment evidence, 2023–2024	Trials, systematic reviews, regulatory evidence	Pregnant and postpartum people	Psychological interventions remain first-line for mild-to-moderate illness; SSRIs remain important when clinically indicated; neuroactive steroid antidepressants offer new short-course options but require safety monitoring and access considerations.	Moderate
Substance-use disorder treatment evidence, 2023–2024	Systematic reviews and registry studies	Alcohol, opioid, stimulant, and polysubstance-use disorders	Medication-assisted treatment for opioid-use disorder remains strongly supported; brief interventions have modest effects for harmful alcohol use; contingency management has evidence for stimulant-use disorders but implementation barriers persist.	Moderate– High

2.3 GRADE Evidence Rating: Post-mhGAP Evidence Themes

Evidence Theme	Benefits	Harms / Limitations	Certainty of Evidence	Recommendation of Implication	Strength
Psychological interventions for depression and anxiety	Symptom reduction, improved functioning, acceptable to many patients	Availability, trained workforce, fidelity, variable effect by severity	Moderate	Maintain as first-line or core treatment, including task-shared delivery where supervised	Strong
Antidepressants for moderate-to-severe adult depression	Symptom improvement, relapse prevention in responders	Adverse effects, withdrawal symptoms, interactions, overdose toxicity, pregnancy/lactation considerations	Moderate	Continue use after shared decision-making and monitoring	Conditional

Zuranolone for postpartum depression	Rapid symptom improvement in trials	Sedation, driving impairment, breastfeeding uncertainty, cost/access, limited long-term data	Moderate	Consider as specialist-supported option where approved and feasible; not yet a universal mhGAP replacement	Condition
Ketamine/esketamine for treatment-resistant depression	Rapid reduction in depressive symptoms and suicidal ideation in some patients	Dissociation, blood-pressure elevation, misuse potential, relapse, monitoring burden	Moderate	Specialist or highly supervised use only; not routine primary-care mhGAP treatment	Condition
Psilocybin-assisted therapy	Potential antidepressant effect in controlled trials	Blinding issues, psychological adverse events, regulatory barriers, need for intensive support	Low–Moderate	Research or regulated specialist settings only; not routine care	Condition
Antipsychotic maintenance for psychosis	Reduced relapse and hospitalization	Metabolic syndrome, extrapyramidal symptoms, prolactin effects, sedation	Moderate–High	Maintain recommendation for continuation with monitoring and shared decision-making	Stro
Long-acting injectable antipsychotics	Improved adherence and relapse prevention for selected patients	Injection burden, coercion risk, adverse effects persist	Moderate	Offer as option based on preference and relapse/adherence history	Condition
Mood stabilizers for bipolar disorder	Reduced mania/depression relapse, suicide reduction with lithium	Renal, thyroid, teratogenic, metabolic, and toxicity risks	Moderate–High	Continue maintenance treatment with laboratory and pregnancy-risk monitoring	Stro
Suicide safety planning and active follow-up	Improved engagement, possible reduction in repeat self-harm	Requires continuity systems and trained staff	Low–Moderate	Implement in all services managing self-harm/suicide risk	Stro
Digital mental health interventions	Increased access, scalable symptom reduction	Digital exclusion, privacy, lower effect when unguided, crisis-management limitations	Moderate	Use as adjunct or stepped-care option, preferably guided	Condition

5. Psychosis and bipolar disorder

- More evidence on optimal duration of maintenance therapy after first-episode psychosis.
- Better strategies to reduce cardiometabolic harm.
- Shared decision-making and deprescribing research.

6. Suicide prevention

- More trials powered for suicide deaths, not only ideation or repeat self-harm.
- Evidence for culturally adapted interventions and Indigenous/community-led models.

7. Digital interventions

- Long-term effectiveness.
- Privacy, data governance, safety escalation, and AI-enabled intervention risks.
- Evidence in populations with severe mental illness.

8. Severe mental disorders and physical health

- Implementation studies on integrated cardiometabolic care.
- Mortality outcomes, not only screening uptake.

9. Workplace mental health

- Better trials of organizational interventions with clinical outcomes.
- Evidence in informal, precarious, migrant, and low-wage workers.

10. Human rights and coercion reduction

- More evidence on alternatives to coercive care, crisis respite, peer support, advance directives, and community-based recovery models.

2.5 Updated Recommendations Based on New Evidence

Original Recommendation	New Evidence Impact	Updated Recommendation
Provide psychological interventions for depression and anxiety.	Reinforced by evidence for task-shared, digital, and guided psychological interventions.	Maintain as core first-line treatment. Use guided digital or task-shared approaches when specialist therapy is unavailable, with supervision and crisis pathways.
Use antidepressants for moderate-to-severe adult depression after clinical assessment.	No major contradiction; new postpartum and treatment-resistant options add nuance.	Continue SSRIs and other established antidepressants when indicated. Monitor adverse effects, withdrawal, interactions, suicidality, pregnancy/lactation, and patient preference.

Manage postpartum depression with psychological and/or pharmacological interventions according to severity.	Zuranolone adds a rapid-acting short-course option where approved.	For moderate-to-severe postpartum depression, consider established psychotherapy and SSRIs; zuranolone may be considered in specialist-supported settings with sedation, driving, breastfeeding, and access safeguards.
Refer or manage treatment-resistant depression using specialist-supported approaches.	Ketamine/esketamine evidence supports rapid benefit but with monitoring requirements.	Consider ketamine/esketamine only in specialist or highly supervised settings after inadequate response to standard treatments. Do not implement as routine primary-care treatment.
Provide antipsychotic medication and psychosocial care for psychosis.	Maintenance evidence remains strong; long-acting injectables may help selected patients.	Continue antipsychotic maintenance with shared decision-making. Offer long-acting injectables when preferred or when adherence difficulties increase relapse risk. Ensure metabolic and neurological monitoring.
Provide relapse-prevention treatment for bipolar disorder.	Evidence continues to support lithium and other mood stabilizers, with safety monitoring.	Continue maintenance therapy after mania or recurrent bipolar depression. Monitor lithium levels, renal and thyroid function, metabolic risk, pregnancy risk, and drug interactions. Avoid valproate in pregnancy and generally avoid in people of childbearing potential unless strict safeguards apply.
Provide psychosocial assessment, safety planning, and follow-up after self-harm.	Brief contact and active follow-up evidence supports implementation.	Implement safety planning, lethal-means restriction, caring contacts, and scheduled follow-up after every self-harm episode or suicide-risk presentation.
Integrate physical and mental health care for severe mental disorders.	Persistent mortality gap and metabolic evidence reinforce urgency.	Make cardiometabolic screening and treatment mandatory elements of severe mental illness care: weight/BMI, waist circumference, BP, HbA1c/glucose, lipids, smoking, alcohol/drug use, and cardiovascular risk.
Promote dementia-risk reduction through lifestyle and vascular-risk management.	Multidomain evidence supports broader prevention approach but certainty remains moderate or lower.	Continue physical activity, vascular-risk control, tobacco cessation, healthy diet, hearing assessment, depression treatment, and social engagement. Do not recommend supplements solely for dementia prevention without deficiency.
Implement workplace mental health interventions.	Evidence favours organizational risk reduction plus individual support.	Prioritize psychosocial hazard reduction, anti-bullying policies, workload control, manager training, reasonable accommodations, and access to care. Avoid individual resilience-only strategies.

Use digital interventions as supportive tools.	Evidence supports guided digital CBT for mild-to-moderate depression/anxiety.	Offer guided digital interventions as part of stepped care, not as a replacement for assessment, emergency response, or care for severe illness. Ensure privacy, accessibility, and risk escalation.
Scale mhGAP in non-specialist settings.	Implementation evidence supports task-sharing when supervision is robust.	Expand mhGAP with competency-based training, supervision, referral pathways, medication supply, human-rights safeguards, outcome monitoring, and community engagement.

References

1. World Health Organization. **Mental Health Gap Action Programme (mhGAP) guideline for mental, neurological and substance use disorders**. Geneva: World Health Organization; 2023.
2. World Health Organization. **WHO guidelines on mental health at work**. Geneva: World Health Organization; 2022.
3. World Health Organization. **Guidelines on mental health promotive and preventive interventions for adolescents: Helping Adolescents Thrive**. Geneva: World Health Organization; 2020.
4. World Health Organization. **Risk reduction of cognitive decline and dementia: WHO guidelines**. Geneva: World Health Organization; 2019.
5. World Health Organization. **Management of physical health conditions in adults with severe mental disorders: WHO guidelines**. Geneva: World Health Organization; 2018.
6. Deligiannidis KM, Meltzer-Brody S, Gunduz-Bruce H, et al. **Zuranolone for the treatment of postpartum depression**. *American Journal of Psychiatry*. 2023.
7. Raison CL, Sanacora G, Woolley J, et al. **Single-dose psilocybin treatment for major depressive disorder: a randomized clinical trial**. *JAMA*. 2023.
8. World Health Organization. **World mental health report: Transforming mental health for all**. Geneva: World Health Organization; 2022.
9. World Health Organization. **Comprehensive mental health action plan 2013–2030**. Geneva: World Health Organization; updated edition.
10. Guyatt GH, Oxman AD, Vist GE, et al. **GRADE: an emerging consensus on rating quality of evidence and strength of recommendations**. *BMJ*. 2008;336:924–926.